

BEFORE THE IOWA BOARD OF SOCIAL WORK EXAMINERS

IN THE MATTER OF:

DELPHINE IRIS JUSTIN
Respondent

)
) DIA NO. 00DPHSW005
)

) FINDINGS OF FACT,
) CONCLUSIONS OF LAW,
) DECISION AND ORDER

On March 11, 2000, the Iowa Board of Social Work Examiners (Board) filed a Statement of Charges against Delphine Iris Justin (Respondent) alleging that she had violated Iowa Code section 147.55(3) and 645 IAC 280.212(3), by engaging in unethical conduct in the practice of social work (Count I) and Iowa Code section 147.55(2) 645 IAC 280.212(2) and 280.213(6), professional incompetency in the practice of social work in the area of record keeping. (Count II). A hearing was scheduled for May 8, 2000. The hearing was rescheduled.

The hearing was held on November 13, 2000 in the fifth floor conference room, Lucas State Office Building, Des Moines, Iowa. The Respondent appeared and was represented by her counsel, Michael Byrne. The state of Iowa was represented by Heather Adams, Assistant Attorney General. The following members of the Board were present for the hearing: James Yeast, L.M.S.W., Chairperson; Jennifer Lock Oman, L.I.S.W.; Lance Clemens, L.I.S.W.; and Joan Hester, Public Member. Margaret LaMarche, Administrative Law Judge from the Iowa Department of Inspections and Appeals, assisted the Board in conducting the hearing. The hearing was closed to the public, at the Respondent's request, pursuant to Iowa Code section 272C.6(1).

The Board convened in closed executive session, pursuant to Iowa Code section 21.5(1)(f)(1999), to deliberate its decision. The Board reconvened for further deliberations on December 11, 2000. The administrative law judge was instructed to prepare the written decision of the Board, in conformance with the deliberations. The Board voted in open session to approve its written decision.

THE RECORD

The record includes the Statement of Charges, Notice of Hearing, Answer; Continuance Orders, Orders Rescheduling the Hearing, the testimony of the witnesses, and the following exhibits:

State Exhibit A:	Letter dated 3/2/93 (Board to Justin)
State Exhibit B:	Complaint, 10/6/98
State Exhibit C:	Investigative Report, 8/30/99
State Exhibit D:	Therapy Session Notes, patient #1

State Exhibit E: Therapy Session Notes, patient #2
State Exhibit F: Curriculum Vitae, Tamara Bench
State Exhibit G: Code of Ethics of the National Association of Social Workers, 3.04
State Exhibit H: Record Keeping, Encyclopedia of Social Work, 19th Edition
State Exhibit I: SOAP Charting Method, The Social Work Dictionary, 4th Edition
State Exhibit J: Children and Families of Iowa, Psycho-Social Assessment Form
State Exhibit K: Children and Families of Iowa, Therapy and Counseling Case Note
State Exhibit L: Children and Families of Iowa, Standard Policies and Procedures
State Exhibit M: Two Rivers Psychological Services, Progress Notes
State Exhibit N: Iowa Code section 232.69
State Exhibit O: Statement of Charges, 3/11/00

Respondent Exhibit 1: Respondent Curriculum Vitae
Respondent Exhibit 2: AP SAC Life Charter Card
Respondent Exhibit 3: SOAP outline
Respondent Exhibit 4: Therapy History Outline
Respondent Exhibit 5: Deposition of Respondent in marriage case
Respondent Exhibit 6: DSM analysis, V61.20
Respondent Exhibit 7: Selected notes on family counseling, typed and handwritten
Respondent Exhibit 8: Therapy session notes, 5/16/97, patient #2
Respondent Exhibit 9: Handwritten note from patient
Respondent Exhibit 10: Therapy session notes, 7/39/97, patient #2
Respondent Exhibit 11: Letter dated 6/18/98 (Respondent to patient #1)
Respondent Exhibit 12: Letter to DHS dated 6/18/98
Respondent Exhibit 13: Attendance, 4/13/96
Respondent Exhibit 14: Certification of Completion of Mandatory Reporter's Training, 4/23/96
Respondent Exhibit 15: Manual
Respondent Exhibit 16: Recognizing and Reporting Child Abuse and Neglect, 1990 Edition
Respondent Exhibit 17: Changes in Child Abuse Legislation Assessments DHS Booklet
Respondent Exhibit 18: Letter from Child Protection Specialists

Respondent Exhibit 19: Iowa Administrative Code, 441 IAC 175
Respondent Exhibit 20: Current abuse reporting booklet received from DHS
Respondent Exhibit 21: NASW reporting requirements; NASW note taking requirements; Ethical Standards
Respondent Exhibit 22: NASW Guidelines on the Private Practice of Clinical Social Work
Respondent Exhibit 23: Diagnosis and Treatment Planning in Counseling, (classroom book) regarding progress notes
Respondent Exhibit 24: Child Maltreatment, Journal of the American Professional Society on the Abuse of Children, May 2000, A Misunderstanding Regarding the Duty To Report Suspected Abuse
Respondent Exhibit 25: Social Work, May 2000, Ethics-R-Us, 197-199
Respondent Exhibit 26: Social Work, May 2000, Ethical Decision Making: The Person In The Process, pp.201-212
Respondent Exhibit 27: Jordan Seminar Material
Respondent Exhibit 28: Letter of Recommendation (Robertson)
Respondent Exhibit 29: Pat Tomson's Curriculum Vitae
Respondent Exhibit 30: Deposition of Pat Tomson
Respondent Exhibit 31: Dr. Rizwan Shah's Curriculum Vitae
Respondent Exhibit 32: Complaint Form
Respondent Exhibit 33: DSM Material-301.83 and 301.50

FINDINGS OF FACT

Respondent Education, Licensure and Employment Background

1. The Respondent received a Master of Arts degree in Social Work at the University of Delhi, India, in 1964. On December 14, 1988, the Respondent was issued license number 304-00583 by the Board to practice as a licensed independent social worker, subject to the laws of the state of Iowa and the rules of the Board. She is a member of the Academy of Certified Social Workers (ACSW), the National Association of Certified Social Workers (NASW), and the American Professional Society on the Abuse of Children (APSAC). (Testimony of Respondent; Respondent Exhibits 1, 2; State Exhibit O)

2. The Respondent has been an individual and family therapist in Iowa since 1982. She maintains a private practice, Counseling

Associates of North Central Iowa, Inc. in Mason City, Iowa. She has no associates in this practice. In addition to her private practice, the Respondent is the clinical director for Parents United of North Central Iowa, a United Way agency which provides services to families and children affected by sexual abuse. The Respondent was one of the founders of this agency in 1983. (Testimony of Respondent; State Exhibit 30)

Child Abuse Reporting Issue

3. On October 8, 1998, the Board received a written complaint against the Respondent from a licensed social worker. The complainant stated that one of her clients, patient #1, had been seen by the Respondent for two and one half years. The complainant stated that patient #1 told her that she had revealed to the Respondent incidents of physical abuse of her children by her husband, but that the incidents were never reported by the Respondent. Patient #1 further stated that she believed that the Respondent documented the abuse but only released a few handwritten notes when asked to turn over her files. (State Exhibit B)

4. The complaint was assigned for investigation and written consent form were obtained from patient #1 and patient #2. Patient #2 is the minor son of patient #1. On July 16, 1999, the investigator met with the Respondent in her office, and the Respondent provided her records for these two patients and additional documents. The Respondent later provided the Board with a typed copy of her handwritten session notes. (Testimony of Respondent; State Exhibits C, D, E)

5. The Respondent began individual therapy with patient #1 on October 20, 1995. Individual therapy continued through May 22, 1998. During this time, the Respondent also had individual and family therapy sessions with the husband of patient #1 and patient #2. On June 18, 1998, the Respondent sent a letter terminating the therapeutic relationship to patient #1. At this time, patient #1 and her husband had separated, divorce proceedings which included child custody issues had been filed, and a child abuse investigation had been initiated. (Testimony of Respondent; State Exhibit C; Respondent Exhibit 11)

6. In his first individual therapy session with the Respondent on May 16, 1997, patient #2 was asked to make a list of his concerns. One of the concerns he listed was that when he was 9 or 10 and lived in Texas, his step father locked him out of the house without his shoes and threw him against a wall and tried to kick him. Patient #2 was fourteen years old at the time of this session.

When the Respondent asked him if anything had happened recently, Patient #2 reported that his room was always messy and one day his stepfather started kicking things and threw a scissors which cut patient #2's leg. Patient #2 reported that this had occurred a couple of days earlier. The Respondent asked him to show her where he was cut. Patient #2 lifted his pants leg and nothing was visible. He told the Respondent "well, it's gone now." The Respondent asked additional questions about what happened and the patient responded that his stepfather was angry about his messy room and started throwing things around. According to the Respondent, the members of this family were all volatile and explosive; throwing things was not uncommon. (Testimony of Respondent; State Exhibits C, D, E)

7. The Respondent testified that she did not report this incident to DHS because she concluded that she did not have a reasonable belief that patient #2 had been physically abused. The Respondent's conclusion was based on the verbal report of patient #2, his demeanor, the absence of any physical injury, and her knowledge of the family dynamics.

According to the Respondent, it would be unusual for an abused child to present such information so directly on a first visit to a therapist. In her opinion, patient #2 did not have the demeanor or the profile of an abused child. He was an honor student, had outside activities, and was socially outgoing. He was hurt that he did not have a relationship with his stepfather which he said he wanted very much. When the Respondent asked if she could tell his stepfather how he felt, patient #2 responded, "tell him." The Respondent felt this would be an unusual response from an abused child.

Based on the description provided by patient #2, the Respondent concluded that she did not have a reasonable belief that the stepfather had intentionally inflicted an injury. The Respondent did not observe any physical injury as a result of the actions of the stepfather. The Respondent did not believe that she was required to report this incident as suspected abuse. (Testimony of Respondent; State Exhibits C, D, E)

8. The Respondent has completed Mandatory Reporter's Training. In her practice, she utilized a Child Abuse and Dependent Adult Abuse Guide for Mandatory Reporters prepared by the Department of Human Services. The Respondent is familiar with the mandatory reporting statute and has made reports regarding suspected physical abuse when she has a reasonable belief that a minor has had a nonaccidental injury. (Testimony of Respondent; Respondent Exhibits 13-20)

9. Dr. Rizwan Shah, M.D. is the medical director of the Child Abuse Program at Blank Children's Hospital in Des Moines, Iowa. She has used the Respondent as a resource for services for abused children. Dr. Shah has worked closely with the Respondent and has full confidence in her ability to understand and properly report abuse. Dr. Shah testified that a professional who is a mandatory reporter should use their training and experience, and not just rely on the report of an alleged victim, in determining whether they have a reasonable belief that abuse has occurred. This would include asking to see the injury if the request would not be invasive. In order to be reportable, an injury must be the result of an intentional act or omission and not accidental. Dr. Shah agreed that it is not the reporter's role to verify abuse.

Dr. Shah reviewed the Respondent's session notes which included references to his allegations of physical abuse by his stepfather. Dr. Shah testified that based on the Respondent's records, she cannot conclude whether it was necessary to report the incident. Dr. Shah testified that if these things were reported to her and she decided not to report it, she would document her reasons. (Testimony of Respondent; Respondent Exhibits 24-28; State Exhibit E)

Recordkeeping Issue

10. On March 2, 1993, the Board sent the Respondent a confidential letter which informed her that a complaint had been filed against her, had been investigated by the Board, and would be closed without formal action. However, the Board notified the Respondent that it had concerns regarding the thoroughness and content of the her case notes. The case notes provided to the Board were found to be inadequate, and they did not conform to generally accepted standards of practice. The Board recommended that the Respondent utilize the "soap" format for her case records. The Board noted that the format simply and efficiently organizes case information to reflect subjective data, objective data, the assessment, and treatment plan. The Board reminded the Respondent that it is important to maintain a case record that accurately reflects the client's contact with the service provider. (Testimony of Respondent; State Exhibit A)

11. The Code of Ethics of the National Association of Social Workers (Revised 1999) addresses client records at section 3.04. It provides, in part, that social workers should take reasonable steps to ensure that documentation in records is accurate and reflects the services provided. Social workers should include sufficient and timely documentation in records to facilitate the

delivery of services and to ensure the continuity of services provided to clients in the future. (State Exhibit G)

12. Standard 8 of The National Association of Social Workers Standards for the Practice of Clinical Social Work (Revised April 1989) provides that clinical social workers shall establish and maintain professional offices and procedures. The social worker keeps records of clients that substantiate service in a safe place. The records are maintained accurately and in a manner that is free from bias or prejudicial content. The records are made available to clients at their request. (Respondent Exhibit 21)

13. The National Association of Social Workers Guidelines on the Private Practice of Social Work (February 1991) also address recordkeeping. While recognizing some variation depending on the social worker's practice and clientele, the guidelines provide that for general purposes, the records should be written in case note or progress note format. Significant activities and intervention throughout the therapeutic process should be documented. For the most part, records should include or address the following areas: identifying information, professional assessment of client, dates of visits, treatment plan (objectives or goals), treatment modality, treatment interventions, treatment outcome, referrals and collaborations, fee information, and a closing note. (Respondent Exhibit 22)

14. Tamara Bench, LISW, testified as an expert witness for the state. Ms. Bench supervises thirteen therapists in her employment with Children and Families of Iowa and is also employed in a private clinical practice in which she provides individual, couples, and family therapy. Ms. Bench testified concerning the standards of practice for recordkeeping by licensed social workers.

Client records must include identifying information (i.e. name, date and time of service, purpose of the therapy, therapist's name); the information or current issues presented by the client on the day of service; subjective and objective observations of the therapist (e.g. client presentation, report of homework); assessment; and a plan of action (treatment goals, client's response to past treatment, plans for next time). In her practice, Ms. Bench uses various printed forms to record and organize this information.

Ms. Bench identified three reasons why these general components must be included in the client record. First, the client may want access to the records and should be able to determine from them what was done in therapy. Second, third parties such as the primary care physician, supervisor, or another therapist may need

access to the notes to determine what has happened in the past and to give the client the best possible service in the future. Third, therapists need to protect themselves by showing what they did for the client and by documenting the client's progress or lack of progress. (Testimony of Tamara Bench; State Exhibits F, J, K, M)

15. The Respondent's records for patient #1 and patient #2 were reviewed by Ms. Bench.

Ms. Bench testified that the Respondent's clinical records for patient #1 and patient #2 do not conform to the standard of care for recordkeeping or to the Code of Ethics or Guidelines for Private Practice established by the National Association of Social Workers. These notes do not include the therapist's intervention; the client's progress; assessment, mental status, or display of symptoms; or the treatment plan for the day or the next session. (Testimony of Tamara Bench; Respondent; State Exhibits C-M; Respondent Exhibit 22)

16. The Respondent testified that in her private clinical practice she keeps a notepad in the client file, in which she writes down what she feels is important during each session. She stated that these notes give her an "emotional content" that is eliminated with the SOAP format. When the Respondent reviews these notes, she is able to place herself back into the client session. The notes serve to prompt the Respondent's memory.

The Respondent conceded that her session notes would be meaningless to someone else. She agreed that she would have to explain them in order for a reader to get any understanding of what occurred in the sessions. The Respondent defended this practice by stating that since she is in a solo clinical practice, no one else is required to review her notes in order to provide services to her clients. The Respondent stated that she has never had a claim for payment denied by a third party payer due to inadequate documentation. The Respondent uses a SOAP type record format in her work for Parents United because the agency has multiple providers and others may need to rely on her records to provide services. (Testimony of Respondent; State Exhibits C, D, E)

17. Pat Thomson is a licensed master level social worker and serves as the Executive Director for Parents United. She has worked with the Respondent at Parents United and has also filled in for the Respondent when she has been absent from her private clinical practice and has consulted with her. Ms. Thompson is familiar with the Respondent's style of note taking in her private practice. She agreed that these records differed from the records maintained at Parents United, which utilizes either a format

requested by DHS or a SOAP format. The format requested by DHS is more detailed than the SOAP format.

Ms. Thompson testified that the Respondent has always been able to provide her with all necessary information regarding the cases that she has consulted on. The Respondent has been able to relate her professional assessment, diagnosis, type of treatment and planning. The Respondent has always been able to provide a letter or written assessment for her clients when requested.

Ms. Thompson consulted with the Respondent and was available to provide services to patient #1 and patient #2 when the Respondent was out of the country. Ms. Thompson did not review the records, but the Respondent discussed the case with her.

Ms. Thompson acknowledged that NASW recommends the SOAP format for recordkeeping. In Ms. Thompson's opinion, however, the Respondent would be able to meet the NASW standard 3.04 for recordkeeping if all of the records and forms she maintained for each patient were reviewed. On cross-examination, Ms. Thompson acknowledged that a social worker's records should include subjective and objective data regarding the client, assessments, and a treatment plan. After reviewing State Exhibits D and E, Ms. Thompson acknowledged that they do not include the Respondent's assessment or treatment plan. The Respondent's assessment was included in a June 18, 1998 letter to DHS, which was drafted after treatment was provided. (Respondent Exhibits 29, 30)

18. In preparation for the hearing, the Respondent prepared SOAP format records and a therapy history outline from her session notes for patient #1 and patient #2, in order to demonstrate that her notes did contain all of the necessary information for adequate records. These records are found at Respondent Exhibits 3 and 4. The Board is unable to determine, based on the Respondent's contemporaneous session notes, whether these records are factual. The Board is left to trust the accuracy of the Respondent's memory for the information and details that were added to complete the record. (Testimony of Respondent; Respondent Exhibits 3, 4, 7-10)

CONCLUSIONS OF LAW

I. COUNT I

Count I charges the Respondent with engaging in unethical conduct in the practice of social work in violation of Iowa Code section 147.55(3) and 645 IAC 280.212(3).

Iowa Code section 147.55(3)(1997) provides, in relevant part:

147.55 Grounds.

A license to practice a profession shall be revoked or suspended when the licensee is guilty of any of the following acts or offenses:

...
3. ...engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

At times relevant to this decision, 645 IAC 280.212(3) provided that the board may impose any of the disciplinary sanctions provided in rule 280.211 when the board determines that the licensee is guilty of engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

Iowa Code section 232.69 (1997) provides, in relevant part:

232.69 Mandatory and permissive reporters-training required.

1. The following classes of persons shall make a report within twenty-four hours and as provided in section 232.70, of cases of child abuse:

...
b. Any of the following persons who, in the scope of professional practice or in their employment responsibilities, examines, attends, counsels, or treats a child and reasonably believes a child has suffered abuse:

- (1) A self-employed social worker.
- (2) A social worker under the jurisdiction of the department of human services.
- (3) A social worker employed by a public or private agency or institution.

"Child abuse" or "abuse is defined, in part, as any nonaccidental physical injury, or injury which is at variance with the history given of it, suffered by a child as the result of the acts or omissions of a person responsible for the care of a child. Iowa Code section 232.68(2)(1997)

The preponderance of the evidence failed to establish that the Respondent engaged in unethical conduct when she did not make a report of abuse to the department of human services following her May 16, 1997 therapy session with patient #2. The Respondent's

conclusion that the she had no reasonable belief that abuse had occurred was not unreasonable, given the patient's description which could be consistent with an accidental injury and the absence of any physical injury a few days following the incident. Based on this record, the Board was unable to conclude that the failure to report abuse was unethical conduct.

II. COUNT II

Count II charged the Respondent with professional incompetency in the practice of social work in the area of recordkeeping, in violation of Iowa Code section 147.55(2) and 645 IAC 280.212(2) and 280.213(6).

Iowa Code section 147.55(2) (1997) provides, in relevant part:

147.55 Grounds.

A license to practice a profession shall be revoked or suspended when the licensee is guilty of any of the following acts or offenses:

- ...
- 2. Professional incompetency.

At times relevant to this decision, 645 IAC 280.212(2) provided that the board may impose any of the disciplinary sanctions provided in rule 280.211 when the board determines that the licensee is guilty of professional incompetence.

645 IAC 280.213(6) provided:

645-280.213(154C) Rules of conduct.

...

280.213(6) General. A licensee shall not:

- ...
- b. Perform services in an incompetent manner.

The preponderance of the evidence established that the Respondent violated Iowa Code section 147.55(2) and 645 IAC 280.212(2) and 280.213(6)"b" when she failed to maintain minimum standards of practice in her recordkeeping for her private practice. The Respondent's clinical records for patient #1 and patient #2 were completely inadequate because they failed to include any objective data, assessments, or treatment plans for these patients.

Minimum standards of practice for recordkeeping by social workers require sufficient, timely, and accurate documentation which reflects the services provided and which facilitates the delivery

of services and ensures continuity of services in the future. Code of Ethics of the National Association of Social Workers, Section 3.04. The National Association of Social Workers Code Guidelines on the Private Practice of Social Work provides an authoritative list of the types of information which should be included in such records. As established by the testimony of the state's expert witness, these standards are recognized and practiced by social workers in the state of Iowa. At a minimum, the social worker's records must include subjective and objective data, assessment, and a treatment plan for the client. As cited by the state's expert, these minimum record keeping standards serve many important purposes.

One of the essential purposes of recordkeeping is to provide continuity of care for the client. The Respondent puts great emphasis her claim that she is the only social worker utilizing her records and that upon request, she can provide all of the required information for each client using her handwritten notes, which trigger her memory. This argument ignores the need for continuity of care. Unless the Respondent supplies the missing pieces, her handwritten notes are meaningless to another reader. The client is forced to rely on the availability of the Respondent and the accuracy of her memory in order to obtain the critical missing information from the record. The standard of care applies regardless of the type of setting in which the licensee practices. There is no exception for social workers in a solo practice.

The Respondent was previously warned by this Board concerning inadequate records. At that time, the Board suggested that the Respondent utilize the SOAP format in her recordkeeping. Despite this direction from the Board, the Respondent continues to omit objective information, assessments, and treatment plans from her records.

III. SANCTION

The Board is authorized to impose the following disciplinary sanctions:

1. Revocation of license.
2. Suspension of license until further order of the board or for a specific period.
3. Prohibit permanently, until further order of the board, or for a specific period, the engaging in specified procedures, methods, or acts.
4. Probation.
5. Require additional education or training.
6. Require a reexamination.

7. Order a physical or mental evaluation, or order alcohol and drug screening within a time specified by the board.
8. Impose civil penalties not to exceed \$1000.
9. Issue a citation and warning.
10. Such other sanctions allowed by law as may be appropriate.

645 IAC 13.1.

The following factors may be considered by the board in determining the nature and severity of the disciplinary sanction to be imposed:

1. The relative serious nature of the violation as it relates to assuring the citizens of this state a high standard of professional care.
2. The facts of the particular violation.
3. Any extenuating facts or other countervailing considerations.
4. The number of prior violations or complaints.
5. The seriousness of prior violations or complaints.
6. Whether remedial action has been taken.
7. Such other factors as may reflect upon the competency, ethical standards, and professional conduct of the licensee.

645 IAC 13.2.

The Board considered these factors in determining an appropriate sanction in this case. Adequate recordkeeping is essential to the competent practice of social work. Although the Board has previously warned the Respondent about deficient recordkeeping, it appears that she did not take the Board's warning seriously, nor did she take remedial actions to correct her recordkeeping practices. The Respondent's testimony in this case suggests that she does not fully understand or appreciate the necessity of timely, complete, and accurate documentation. For these reasons, the Board believes that the Respondent's recordkeeping violations must be addressed through a period of probation with supervision and monitoring of her recordkeeping.

ORDER

IT IS THEREFORE ORDERED, that the social work license issued to Delphine Justin, license no. 304-00583, is hereby placed on probation for an indefinite period, subject to the following terms and conditions:

1. Within thirty (30) days of the issuance of this order, the Respondent shall submit a written plan of supervision to the Board for its approval. The plan shall include the names

and curriculum vitae of three proposed supervisors. The proposed supervisors shall be licensed independent social workers. One will be selected by the Board to serve as a practice supervisor for the Respondent. The approved supervisor will be provided a copy of the Board's Findings of Fact, Conclusions of Law, Decision and Order in this case.

The plan of supervision must include review of the Respondent's records for compliance with the standards of practice relating to recordkeeping and specifically for compliance with the NASW Guidelines on the Private Practice of Clinical Social Work. The supervisor shall provide quarterly written reports to the Board indicating the Respondent's compliance or any lack of compliance. All costs associated with the supervision shall be the responsibility of the Respondent.

2. After six (6) months of social work practice under supervision, the Respondent shall make a personal appearance before the Board and present evidence that she has been in compliance with this decision and order. At that time, the Board will determine whether to continue the period of probation.

IT IS FURTHER ORDERED, that the Respondent shall pay a civil penalty of two hundred and fifty dollars (\$250).

IT IS FURTHER ORDERED, pursuant to Iowa Code section 272C.6 that the Respondent shall pay \$75.00 for fees associated with the disciplinary hearing within thirty (30) days of receipt of this decision. Any appeal to the district court from disciplinary action of the board shall be taken within thirty (30) days from the issuance of the decision by the board. It is not necessary to request a rehearing before the board to appeal to district court. 645 IAC 11.25. If an appeal is filed in district court, any costs incurred for the transcription of the record shall be paid by the Respondent.

Dated this 26th day of March, 2001.


James Yeast, L.M.S.W.
Chairperson
Iowa Board of Social Work Examiners

DIA No. 00DPHSW005

Page 15

cc: Heather Adams
Assistant Attorney General

Michael Byrne
Attorney for Respondent

BEFORE THE IOWA BOARD OF
SOCIAL WORK EXAMINERS

IN THE MATTER OF:	)	CASE NUMBER: 98-11
	)	
DELPHINE IRIS JUSTIN,	)	STATEMENT OF CHARGES
	)	
Respondent.	)	

TO THE ABOVE NAMED RESPONDENT:

The Iowa Board of Social Work Examiners has jurisdiction of this matter pursuant to Iowa Code chapters 17A, 154C, and 272C (1997).

Licenses issued by the Board are subject to the laws of the state of Iowa and to the administrative rules of the Board.

Respondent Delphine Iris Justin was at all times during the following events a licensed independent social worker. Respondent was issued license number 304-00583 to practice as a social worker in the state of Iowa on December 14, 1988.

Respondent's current address as reported to the Board is 1750 Springview Drive, Mason City, Iowa, 50401.

COUNT I

Respondent is charged pursuant to Iowa Code section 147.55(3) and 645 Iowa Administrative Code 280.212(3) with engaging in unethical conduct in the practice of social work.

CIRCUMSTANCES

1. Respondent provided social work services to patient # 1 from October of 1995 to May of 1998. Respondent provided social work services to patient # 2, the minor child of patient # 1, from May of 1997 to August of 1997.

2. In May of 1997, patient # 2 informed Respondent that he had been physically abused by his step-father. Patient # 2 provided detailed information regarding the physical abuse to Respondent. Respondent documented this information in the case notes of patient # 2.

3. Respondent is required by Iowa Code section 232.69 to make a report to the department of human services if in the scope of her professional practice she counsels or treats a child and reasonably believes the child has suffered abuse.

4. Respondent failed to make a report to the department of human services regarding the physical abuse reported by patient # 2.

COUNT II

Respondent is charged pursuant to Iowa Code section 147.55(2) and 645 Iowa Administrative Code 280.212(2) and 645 Iowa Administrative Code 280.213(6) with professional incompetency in the practice of social work in the area of record keeping.

CIRCUMSTANCES

1. Respondent provided social work services to patient # 1 from October of 1995 to May of 1998. Respondent provided social work services to patient # 2, the minor child of patient # 1, from May of 1997 to August of 1997.

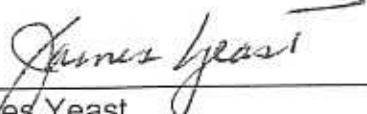
2. Respondent's case notes with respect to patients # 1 and # 2 are incomplete and inadequate. The case notes do not include objective data, assessments, or treatment plans.

3. Respondent had been previously warned by the Board that her case notes did not conform to accepted standards of practice due to the lack of content and thoroughness of her record keeping.

FINDING OF PROBABLE CAUSE

On January 21, 2000, the Iowa Board of Social Work Examiners found probable cause to file this Statement of Charges and to order that a hearing be set in this case.

Executed this 11th day of March, 2000.



James Yeast
Iowa Board of Social Work Examiners
Board Chairperson